

August 12, 2026

South Carolina Medicaid (Healthy Connections / SCDHHS)
Provider Appeals Department

RE: Formal Appeal of Claim Denial — Renato Dare (MRN MUSC1793698), Claim MUSC-20260110-FFD5-2

Patient	Renato Dare (DOB 1976-06-14)
MRN	MUSC1793698
Member ID / Group	SCD346795729 / GRP-86928
Claim number	MUSC-20260110-FFD5-2
Date of service	2026-01-10
Procedure billed	CPT 99214 — Office/outpatient visit, established patient, moderate complexity
Primary diagnosis	J06.9 — Acute upper respiratory infection, unspecified
Denied amount	\$420.93
Denial code	CARC 11 / RARC M64 — The diagnosis is inconsistent with the procedure.
Appeal deadline	2026-03-25

Dear Appeals Review Committee:

SUMMARY OF APPEAL

MUSC Health Florence Medical Center submits this first-level appeal on behalf of patient Renato Dare (MRN: MUSC5811662, Member ID: SCD346795729) regarding the denial of Claim MUSC-20260110-FFD5-2 for a date of service of January 10, 2026. The claim was submitted on January 29, 2026, and denied on February 23, 2026, under CARC 11 ("The diagnosis is inconsistent with the procedure") and RARC M64 ("Missing/incomplete/invalid other diagnosis"), with the payer remark stating: "The diagnosis code reported does not support the procedure billed." The total denied amount is \$420.93. We respectfully contend that this denial is not supported by the clinical record and that the claim should be reprocessed and paid at the allowed amount of \$305.55.

CLINICAL BACKGROUND

Mr. Dare is a 50-year-old male established patient with a documented, active chronic disease burden that includes diabetes (ICD-10 E11.9, onset 2001), diabetic neuropathy (onset 2007), metabolic syndrome (onset 2007), hypertriglyceridemia (onset 2004), hyperglycemia (onset 2010), prediabetes (onset 2010), and anemia (ICD-10 D64.9, onset 2001). He carries an active medication regimen that includes extended-release metformin, initiated in 2007 for his diabetes. On January 10, 2026, Dr. Elena V. Castellanos, MD (NPI 1881726354), evaluated Mr. Dare at MUSC Health Florence Medical Center in an on-campus outpatient hospital setting. The primary diagnosis reported for that encounter was J06.9 (Acute upper respiratory infection, unspecified), with a secondary diagnosis of R69. The service billed was CPT 99214, an established-patient office or outpatient visit of moderate complexity.

BASIS FOR APPEAL

The payer's denial rests on the assertion that J06.9 does not support CPT 99214. This position does not accurately reflect CPT coding guidance. CPT 99214 is defined by the level of medical decision-making or time involved in the encounter, not by any single diagnosis code. An established patient with an acute presenting complaint such as an upper respiratory infection, evaluated in the context of multiple active chronic conditions including diabetes with neuropathy, metabolic syndrome, hypertriglyceridemia, and anemia, presents a clinical picture that routinely meets the threshold for moderate complexity medical decision-making. The management of an acute illness in a patient whose chronic conditions may affect treatment selection, drug interactions, and clinical course — for example, the use of antibiotic therapy in a diabetic patient with neuropathy and metabolic comorbidities — inherently involves a greater degree of clinical judgment than the same complaint in an otherwise healthy individual.

Regarding RARC M64, the secondary diagnosis of R69 was submitted on the claim and is supported by Mr. Dare's active documented conditions of hyperglycemia, metabolic syndrome, and hypertriglyceridemia, all coded to R69 in his longitudinal record. The diagnosis information submitted was neither missing nor invalid; it was present and consistent with the patient's known medical history. To the extent the payer requires additional specificity, the clinical record supports the submission as made, and any perceived coding gap does not negate the medical necessity or the appropriateness of the visit level.

South Carolina Medicaid's own coverage policies recognize that established-patient evaluation and management services are covered when medically necessary and appropriately documented. The encounter on January 10, 2026, meets that standard given Mr. Dare's clinical complexity.

REQUESTED ACTION

MUSC Health respectfully requests that South Carolina Medicaid (Healthy Connections / SCDHHS) overturn the denial of Claim MUSC-20260110-FFD5-2, reprocess the claim, and issue payment at the previously determined allowed amount of \$305.55 for CPT 99214 rendered on January 10, 2026. We are prepared to submit the complete encounter documentation upon request and welcome the opportunity to provide any additional information required to resolve this matter promptly.

Respectfully submitted,

Angela R. Whitfield, RN, BSN, CPC

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Enclosures

- Remittance advice / explanation of payment
- Itemized claim detail (UB-04 / CMS-1500)
- Relevant clinical documentation from the medical record